Initial Observation

During the initial meeting, Zubair appeared weak but maintained good eye contact. He seemed confused, often pausing before responding. He was talkative but exhibited manipulative behaviors. Discussing his family made him visibly sad, though he claimed to be satisfied with his life.

Developmental History of the Problem

Zubair's issues began seven years ago following a significant business loss and betrayal by friends. Anger issues led to conflicts with his family and a brief jail term. Substance use began at age 20 as a means of coping, which escalated over time.

Background Information

Personal History

Zubair managed a utility store and lives in a joint family. He married five years ago and has a four-year-old son. He experienced the loss of a daughter, which contributed to his stress.

Premorbid Personality

Before substance use, Zubair led a fulfilling life, maintained good relationships, and was generally not judgmental, though he did have anger issues.

Family History

Zubair’s father, [Father's Name], has a business and is authoritative. He has five siblings. His family dynamics are generally positive, although his father's approach can be controlling.

Educational History

Zubair started school at age 5 and completed a diploma in Electrical Engineering by age 23.

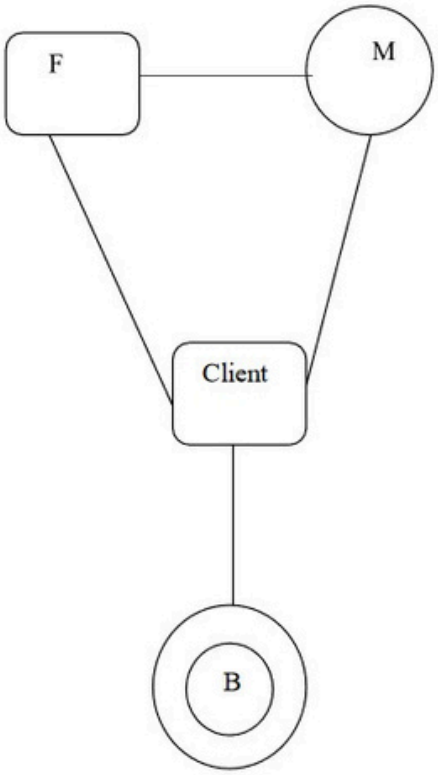
Occupational History

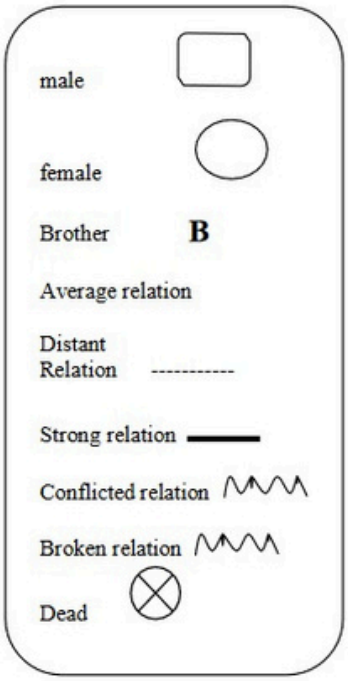
Zubair owned a business selling fans and worked in a utility store.

Drug History

Zubair has been using heroin and ice regularly with friends. His family was unaware until recently, prompting intervention.

Figure 1 Genogram





Assessment

The assessment was carried out using multiple modalities including:

Behavioral Observation

Clinical Interview

Mental Status Examination

Subjective Ratings

Personal Hygiene Checklist

Indigenous Anger Scale

Behavioral Observation

Zubair appeared weak but well-dressed. He was initially resistant but became cooperative. His emotional state fluctuated, especially when discussing family.

Clinical Interview

The interview revealed a history of strong family ties and social interactions, alongside his struggles with substance abuse and aggression.

Subjective Ratings

Problems	Subjective
Addiction	Ratings 6/10
Insight	7/10 7/10
Anger	

Mental Status Examination

Areas	Status
Memory	Appropriate
Insight	Present
Speech	Disorganized
Appearance	Neat and clean

Indigenous Anger Scale

Mean	Standard Deviation	Obtained Score
38.3	13.75	79.75

0

Case Formulation

Using the biopsychosocial model:

Predisposing Factors:No genetic predisposition noted.

Precipitating Factors: Loss of his father and social environment.

Perpetuating Factors: Avoidance behaviors maintaining the disorder.

Protective Factors: Supportive family dynamics.

Diagnosis

292.0 (F19.239) Substance Withdrawal

V62.89 (Z60.0) Phase of Life Problem

Therapeutic Techniques

Rapport Building: Establishing trust for effective therapy.

Psychoeducation: Educating Zubair about addiction and its consequences.

Anger Management Techniques: Strategies to cope with anger.

Craving Management: Identifying triggers and coping mechanisms.

Relaxation Exercises: Techniques for physical and mental relaxation.

Case Formulation for Zubair Ahmad

Client Profile:
Zubair Ahmad is a 29-year-old male from a middle-class family. He is educated but exhibits significant behavioral issues, including frequent outbursts of anger, aggression toward others (excluding family), and a pattern of substance use (heroin and ice). His tendency to roam at night and stubbornness further complicate his situation.

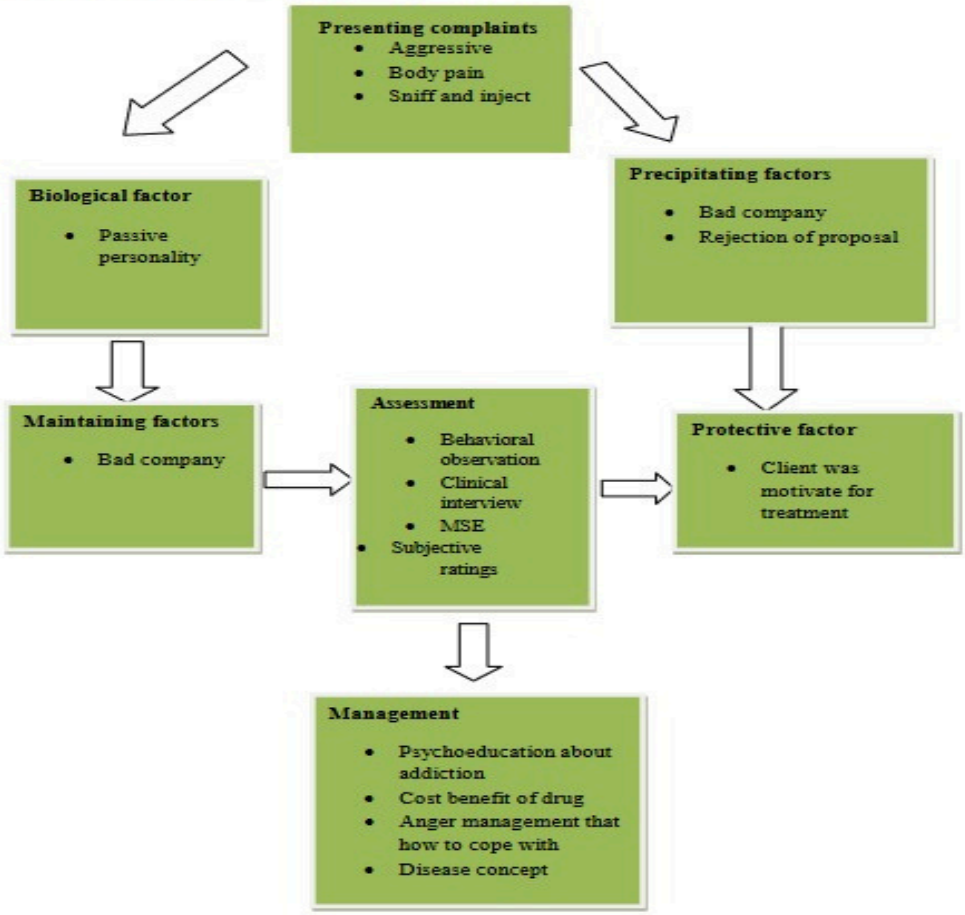
Biopsychosocial Model

1. Predisposing Factors:
   - Genetic Background: There are no identifiable genetic factors contributing to Zubair's conduct issues, indicating that his problems do not stem from hereditary influences.
2. Precipitating Factors:
   - Social Environment: Zubair's aggressive behaviors and substance use are largely influenced by his social interactions and peer pressure.
   - Traumatic Loss: Past traumatic experiences, such as the loss of significant relationships, may have triggered emotional instability and manipulative behaviors associated with conduct disorder (Nell, 2022).
3. Perpetuating Factors:

- Avoidance Behavior: Zubair displays avoidance behaviors that hinder his recovery process. Research indicates that such patterns can exacerbate conduct disorder symptoms and maintain negative cycles (Chou, 2009).
- Continued Substance Use: His ongoing drug use reinforces these behaviors and diminishes motivation for positive change.

4. Protective Factors:
   - Family Support: The caring nature of Zubair's family serves as a protective factor, potentially alleviating some negative influences related to his behavior. Studies suggest that positive family relationships can significantly enhance recovery prospects for individuals with conduct disorders (Stone et al., 2015).

Summary of the Case Formulation



Session Reports

Session 1: Initial Assessment (18-06-24)

Zubair arrived appearing nervous but maintained good eye contact. He seemed reluctant to discuss his history.

Conducted a clinical interview focusing on his family dynamics, educational background, and substance use history.

Zubair was initially quiet and hesitant but began to open up about his family relationships. He expressed feelings of guilt regarding his substance use and its impact on his family.

Established rapport; gathered initial data for assessment.

Session 2: Exploring Family Dynamics (21-06-24)

Zubair appeared more relaxed and engaged. He shared more about his family, including the impact of losing his daughter.

Discussed his family relationships in depth, including conflicts and support systems. Explored his feelings around his daughter's loss and how it relates to his substance use.

Zubair showed increased emotional engagement, particularly when discussing his daughter. He began to acknowledge the negative impact of his behavior on his family.

Deeper understanding of family dynamics and emotional triggers; began identifying coping strategies.

Session 3: Mental Status Examination and Anger Assessment (23-06-24)

Conducted Mental Status Examination (MSE) and the Indigenous Anger Scale.

Administered MSE, assessing appearance, mood, speech, and cognitive functioning. Discussed anger triggers and responses.

Zubair displayed some signs of disorganization in thought, particularly when discussing his anger. His responses to anger-related questions revealed significant distress.

MSE indicated low mood and high anger levels; identified specific triggers for his aggressive behavior.

Session 4: Interventions and Goal Setting (25-06-24)

Zubair showed notable improvement in mood and engagement.

Implemented psychoeducation regarding addiction and anger management techniques. Discussed relaxation exercises and coping strategies for cravings.

Zubair was more optimistic and receptive to feedback. He actively participated in discussions about strategies to cope with cravings and anger.

Developed a clear set of coping strategies; established goals for future sessions, focusing on maintaining engagement with family and avoiding substance use.

Outcome

Post-assessment showed significant improvement in Zubair's anger management, insight, and personal hygiene.

Pre and Post Assessment Summary

Pre-assessment showed 0% in basic hygiene tasks; post-assessment revealed marked improvement (e.g., brushing teeth at 90%). Insight increased from 5% to 7% regarding addiction and personal impact. Anger levels decreased from 100% to 30%, indicating effective anger management strategies.

References

Diagnostic and Statistical Manual of Mental Disorders (5th ed.). Arlington, VA: American Psychiatric Publishing.
anual for the Beck Depression Inventory. San Antonio, TX: Psychological Corporation.

Cognitive Behavior Therapy: Basics and Beyond (2nd ed.). New York: Guilford Press.

Clinical Psychology: A Very Short Introduction. Oxford: Oxford University Press.

ICD-11 for Mortality and Morbidity Statistics (ICD-11 MMS). Geneva: World Health Organization.

Cognitive-Behavioral Treatment of Borderline Personality Disorder. New York: Guilford Press.